

Cesarean section

A Cesarean section is an operation performed by an obstetrician to deliver a baby. The procedure is considered major abdominal surgery and involves the use of anesthesia.

A Cesarean section is the delivery of your baby through an incision in your abdomen. A Cesarean rate of around 10–15 percent is strived for, although in most Western countries the rate has risen beyond 20 percent, and around 30 percent of all babies in the US are now delivered by Cesarean—an all-time high.

An increase in Cesareans

There are several reasons for the rise in the number of Cesarean births. Women are choosing to have their babies when they are older and are therefore having fewer babies. This makes them more risk-averse, so if there is any likelihood of there being complications, they might opt for a Cesarean. There has been an increase in the number of pregnant women who have a high BMI (see p.16) and this is associated with an increased chance of Cesarean birth. Women who have had a previous Cesarean may choose to have another, as might those who had a previous traumatic delivery. A woman who has had one or two previous Cesarean should be offered a trial of labor, known as [Vaginal Birth After Cesarean \(VBAC\)](#), but may choose to have a planned Cesarean. This decision will be influenced by her previous birth history and whether she is prepared to accept the risks associated with laboring after a previous Cesarean section. A woman who has had three or more Cesareans is recommended to have a repeat Cesarean because there is an increased risk in labor to the mother and baby.

A small but increasing number of women do not wish to labor and therefore opt to have a Cesarean. Your obstetrician will discuss with you the risks and benefits of having a Cesarean delivery so that you can make an informed decision.

At full term, 2 percent of babies are lying in the breech position and it is generally thought to be safer for a breech baby to be born by Cesarean section. A Cesarean is also safer for other conditions, such as a placenta previa (see [Low-lying placenta](#)) or fibroids.

Emergency and urgent Cesareans

A woman might need a Cesarean section for many reasons, and there are differing degrees of urgency. An emergency Cesarean is carried out when there is an immediate threat to the baby's or mother's life. An urgent Cesarean is when there is concern for the baby's or mother's well-being, but no immediate threat to life. A scheduled Cesarean is when there is no immediate concern for the mother or the baby, but an early delivery is advised, perhaps because of a condition in the mother or baby. An elective Cesarean is scheduled by the patient's preference, after 39 weeks (see [An elective Cesarean](#)).

An emergency Cesarean is most commonly carried out when the baby is thought to be at risk—for example, if the baby's oxygen supply has been reduced and there are signs of fetal distress. In this case, the staff will try to do the Cesarean rapidly—within 30 minutes, when possible—although it's important that no shortcuts are taken that could put the mother's own health at unnecessary risk.

Urgent Cesareans take place when there's no immediate threat to life, and these are more common than emergency Cesareans. An urgent Cesarean may be done if the baby's heart rate is causing concern very early in labor and the prospect of a vaginal delivery taking place within a reasonably short time is low. An example of a scheduled Cesarean is one that is done because of a failed induction of labor.